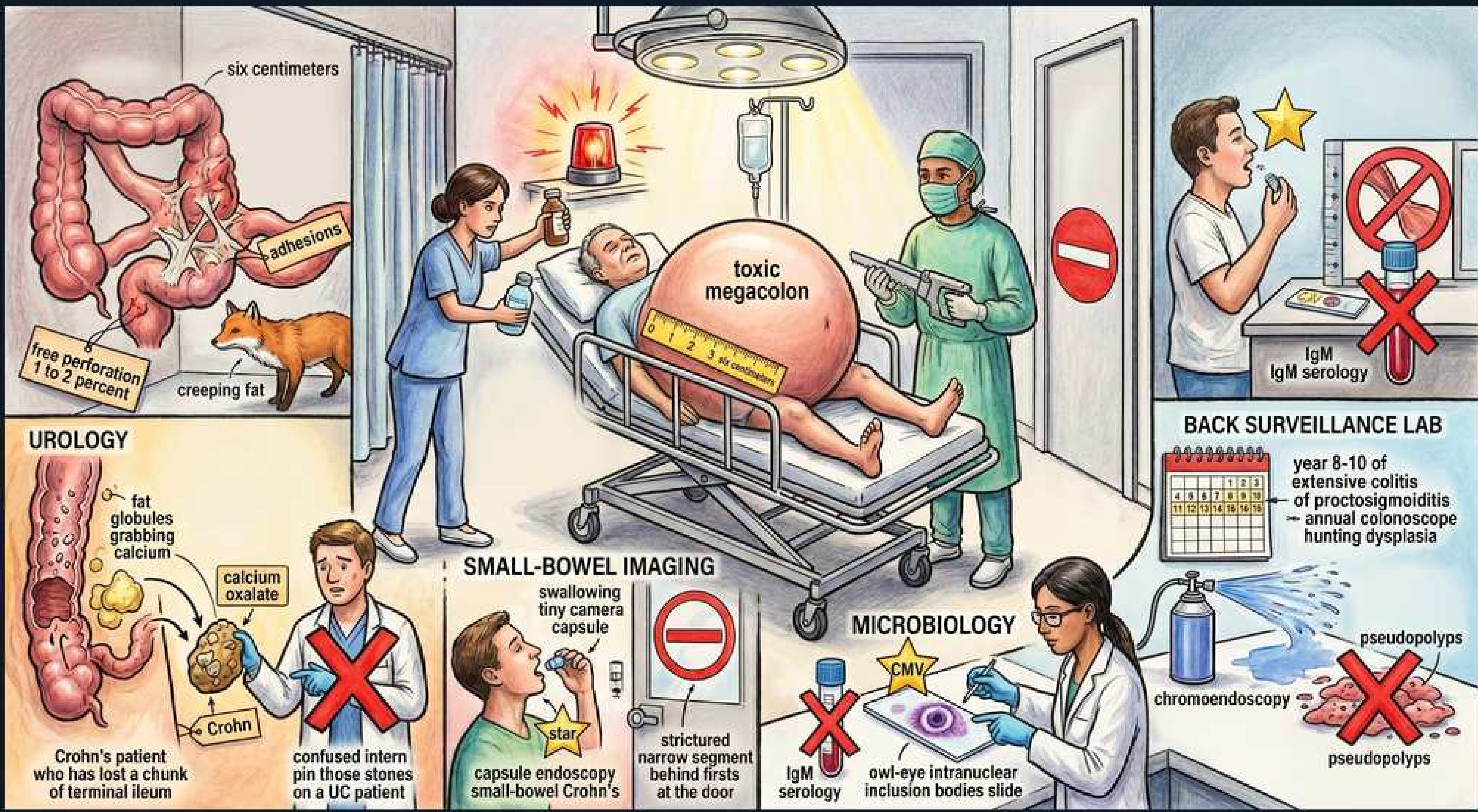
IBD — Complications & Surveillance



1. Toxic megacolon (distended belly)

WHAT YOU SEE

Patient on a gurney with a hugely distended belly, alarm light, labeled 'TOXIC MEGACOLON'.

WHAT IT ENCODES

Toxic megacolon = transverse colon dilation >6 cm PLUS systemic toxicity (fever, tachycardia, leukocytosis, hypotension). Emergency: IV steroids, bowel rest, broad antibiotics; surgery if no improvement in 24–72 h.

2. Six centimeters colon

WHAT YOU SEE

Dilated colon at top-left labeled 'SIX CENTIMETERS'.

WHAT IT ENCODES

The diagnostic radiographic threshold for toxic megacolon is transverse colon diameter >6 cm on plain film. Above this with toxicity defines the emergency.

3. Avoid opioids/anticholinergics (stop signs)

WHAT YOU SEE

Red 'no-entry / stop' signs around the bedside and surgeon.

WHAT IT ENCODES

In toxic megacolon AVOID opioids, anticholinergics, and antidiarrheals (e.g., loperamide) — they paralyze the colon and worsen dilation/perforation risk. The stop signs flag contraindicated motility-slowing drugs.

BOARD TRAP

"Give antidiarrheals/opioids to control toxic megacolon" → wrong; they worsen dilation and risk perforation.

4. Free perforation 1–2% (creeping fat/fox)

WHAT YOU SEE

Label 'free perforation 1 to 2 percent' with a fox and 'creeping fat'.

WHAT IT ENCODES

Free perforation complicates ~1–2% of IBD, more in Crohn's (transmural). Presents with peritonitis and free air — surgical emergency. Creeping fat marks the transmural Crohn's context.

5. Calcium-oxalate stones — Crohn's (urology)

WHAT YOU SEE

Urology panel: bowel losing a chunk, 'fat globules grabbing calcium', 'CALCIUM OXALATE', labeled 'Crohn'.

WHAT IT ENCODES

Terminal-ileal Crohn's → fat malabsorption → luminal fat binds calcium → free oxalate is absorbed → enteric hyperoxaluria → calcium-OXALATE kidney stones. Mechanism is unique to fat-malabsorbing small-bowel Crohn's.

6. Confused intern pins stones on UC (red X)

WHAT YOU SEE

Bottom-left intern crossed with red X 'confused intern... pin those stones on a UC patient'.

WHAT IT ENCODES

FALSE: calcium-oxalate stones are a CROHN'S (terminal-ileal malabsorption) complication, NOT UC. UC spares the small bowel, so enteric hyperoxaluria does not occur — a board trap.

BOARD TRAP

"Calcium-oxalate stones occur in ulcerative colitis" → wrong; they are a terminal-ileal Crohn's complication.

7. Capsule endoscopy small-bowel Crohn's (gold star)

WHAT YOU SEE

Patient swallowing a tiny camera, gold star, 'capsule endoscopy small-bowel Crohn's'.

WHAT IT ENCODES

Capsule endoscopy is excellent for evaluating SMALL-BOWEL Crohn's not reached by colonoscopy. Gold star = preferred small-bowel imaging when no stricture risk.

8. Patency capsule first (stop sign)

WHAT YOU SEE

Red 'no-entry' sign with 'stricture narrow segment behind first' near the door.

WHAT IT ENCODES

In suspected strictures, use a dissolvable PATENCY capsule FIRST — a real video capsule can lodge above a stricture and cause obstruction needing surgery. Confirm patency before the camera capsule.

BOARD TRAP

"Send the video capsule without checking for strictures" → wrong; a stricture can trap it; use a patency capsule first.

9. CMV owl-eye inclusions (gold star)

WHAT YOU SEE

Microbiology panel: 'CMV', 'owl-eye intranuclear inclusion bodies slide', gold star.

WHAT IT ENCODES

CMV colitis (suspect in steroid-refractory IBD flares) is diagnosed by tissue biopsy showing owl-eye intranuclear inclusion bodies (and immunohistochemistry/PCR). Gold star = correct diagnostic method. Treat with ganciclovir.

10. CMV IgM serology crossed (red X)

WHAT YOU SEE

Microbiology slide crossed 'IgM serology' with red X.

WHAT IT ENCODES

FALSE: diagnosing CMV colitis by IgM serology. Serology cannot confirm tissue-invasive CMV — diagnosis requires BIOPSY (owl-eye inclusions / IHC / tissue PCR). Serology is a trap.

BOARD TRAP

"Diagnose CMV colitis with IgM serology" → wrong; need tissue biopsy with owl-eye inclusions, not serology.

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11. Surveillance year 8–10 / chromoendoscopy

WHAT YOU SEE

Back surveillance lab: calendar 'year 8-10 of extensive colitis... annual colonoscope hunting dysplasia', 'chromoendoscopy'.

WHAT IT ENCODES

Colorectal cancer surveillance begins 8–10 years after onset of extensive/long-standing colitis (earlier and annually if PSC coexists), with annual colonoscopy using chromoendoscopy and targeted biopsies for dysplasia.

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12. Pseudopolyps not cancer (red X)

WHAT YOU SEE

Bottom-right pink polyp clusters labeled 'pseudopolyps' with red X.

WHAT IT ENCODES

Pseudopolyps are regenerative mucosal islands from prior ulceration — they are NOT malignant. Mistaking pseudopolyps for cancer is a board trap; true dysplasia/neoplasia is what surveillance targets.

BOARD TRAP

"Pseudopolyps are malignant lesions" → wrong; they are benign regenerative tissue, not cancer.

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13. Right surveillance patient (gold star)

WHAT YOU SEE

Top-right healthy patient with gold star at the surveillance lab, IgM serology crossed beside him.

WHAT IT ENCODES

Correct surveillance pathway: scheduled colonoscopic dysplasia screening with chromoendoscopy is the right move (gold star), versus relying on serology or treating pseudopolyps as cancer.
